EHRA
HIMSS ELECTRONIC HEALTH RECORD ASSOCIATION

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June 16, 2025

Mehmet Oz, M.D.
Administrator, Centers for Medicare & Medicaid Services
U.S. Department of Health and Human Services
7500 Security Boulevard
Baltimore, MD 21244

Re: Health Technology Ecosystem Request for Information; Docket Number: CMS-0042-NC

Dear Administrator Oz,

On behalf of our nearly 30 member companies that develop, implement, and support certified health IT for the vast majority of hospitals and ambulatory providers nationwide, the HIMSS Electronic Health Record (EHR) Association appreciates the opportunity to submit comments collectively to CMS, ASTP/ONC, and HHS in response to the joint Request for Information on the Health Technology Ecosystem.

Since our founding in 2004, the EHR Association has served as a collective voice for the developer community, working closely with ONC/ASTP, CMS, and other federal partners to help realize the goals of interoperability, usability, and value-based care. We are committed to enhancing the quality and efficiency of care through the adoption and utilization of innovative, interoperable health IT. As such, we are committed to working towards a healthcare ecosystem that leverages the capabilities of EHRs and other health IT to deliver higher-quality care to patients efficiently and in a productive and sustainable manner.

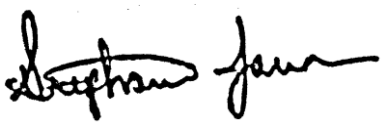
We share the agency's belief that the practical and responsible adoption of health IT can empower patients to make better decisions about their health and overall well-being. We appreciate the opportunity to share our insights on the state of data interoperability and the broader health technology infrastructure, as well as digital health products for patients and caregivers.

Our detailed comments follow. We look forward to continued collaboration with CMS and ASTP/ONC toward improved patient care.

AdvancedMD
Altera Digital Health
Athenahealth
BestNotes
CureMD
eClinicalWorks
Elation Health
Elekta
EndoSoft
Epic
Experity
Flatiron Health
Foothold Technology
Greenway Health
Harris Computer /MEDHOST
MatrixCare
MEDITECH, Inc.
Modernizing Medicine
Netsmart
Nextech
NextGen Healthcare
Office Practicum
Oracle Health
PointClickCare
Sevocity
TruBridge
Veradigm

Sincerely,

Leigh Burchell
Chair, EHR Association
Altera Digital Health



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Vice Chair, EHR Association
Greenway Health

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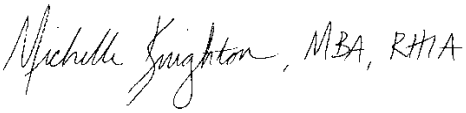
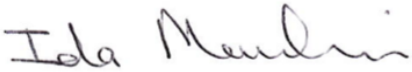


Michelle Knighton
NextGen Healthcare



Ida Mantashi
Modernizing Medicine

Shari Medina, MD
Harris Healthcare

Kayla Frederickson
Oracle Health

Established in 2004, the Electronic Health Record (EHR) Association is comprised of 27 companies that supply the vast majority of EHRs to physicians' practices and hospitals across the United States. The EHR Association operates on the premise that the rapid, widespread adoption of EHRs will help improve the quality of patient care as well as the productivity and sustainability of the healthcare system as a key enabler of healthcare transformation. The EHR Association and its members are committed to supporting safe healthcare delivery, fostering continued innovation, and operating with high integrity in the market for our users and their patients and families. The EHR Association is a partner of HIMSS. For more information, visit www.ehra.org.

Patients and Caregivers

PC-5a. What role, if any, should CMS have in reviewing or approving digital health products on the basis of their efficacy, quality, or impact, or both on health outcomes (not approving in the sense of a coverage determination)? What criteria should be used if there is a review process? What technology solutions, policy changes, or program design changes can increase patient and caregiver adoption of digital health products (for example, enhancements to data access, reimbursement adjustments, or new beneficiary communications)?

The term "digital health products" is broad and could encompass EHRs, certified health IT, consumer apps, and tools used by community-based organizations. CMS should not duplicate ASTP/ONC's existing certification program, which already ensures baseline functionality, interoperability, and security for EHRs and related systems. Any CMS role in evaluating other digital tools should be clearly scoped and avoid regulatory overlap. Adoption is best supported through expanded data access, improved digital literacy, and alignment of incentives across programs.

PC-5b. What changes would enable timely access to high-quality CMS and provider-generated data on patients?

One important area worth effort from all sides is patient matching. Timely access to high-quality CMS and provider-generated data relies on improved patient matching across systems and among stakeholders. CMS should prioritize national strategies to enhance patient identity resolution. Digital identity management plays a crucial role in this, but reliable matching and linking to the actual record is the most critical aspect to ensure accurate access to the patient's complete record across data holders. Additionally, requirements relating to standards adoption for health data exchange should be extended to ancillary providers (e.g., labs, imaging centers) and payers to ensure data completeness and interoperability across the entire ecosystem.

PC-7. If CMS were to collect real-world data on digital health products ' impact on health outcomes and related costs once they are released into the market, what would be the best means of doing so?

Measuring real-world cost impacts may be feasible through claims and beneficiary expense data, but measuring data and drawing conclusions regarding health outcomes remains a long-standing challenge. As clinicians will surely convey in their responses to the RFI and as is well-studied in the context of non-medical influences on patient health, numerous factors contribute to patient care that are unrelated to the discussions and decisions made within the care delivery system, and there is genuine complexity in attributing outcome results to individual providers or even teams.

We note again that "digital health products" is a large category of software. However, if the intent here is specifically to draw connections between app usage and health outcomes or costs, we believe a key challenge is the lack of quality measures specific to that category, much less robust measures that

provide an opportunity to report not only for primary care but also for an array of specialties. Without standardized outcome measures or a certification framework for digital apps, comparisons across products are likely to be inconsistent and unreliable. CMS would need to address the lack of uniform standards in app environments to enable any type of meaningful evaluation.

PC-8. In your experience, what health data is readily available and valuable to patients, their caregivers, or both?

Traditional clinical information, such as medications, lab results, and visit summaries, is generally available to patients and caregivers; however, access may not always be as seamless or centralized as needed. The information blocking regulation and assorted certification requirements have dramatically expanded the data that now flows from EHRs to patient portals and apps, but it is clear that there are still challenges presented by portals that patients log into separately and the fact that connections between EHRs and API-based data sources are still relatively limited in their uptake. CMS has an opportunity to incite more commitment from provider organizations to installing available technologies, such as enabling and maintaining HL7 FHIR-based API endpoints, as well as the effort that is required to really work with patients on establishing connections to the digital technologies of their choice.

PC-8a. What data is valuable, but hard for patients and caregivers, or app developers and other technical vendors, to access for appropriate and valuable use (for example, claims data, clinical data, encounter notes, operative reports, appointment schedules, prices)?

Non-medical factors that strongly influence health, such as food security, transportation access, and family support, are rarely captured in structured, exchangeable formats. Even when this data is collected by community-based organizations, it is not routinely shared with patients or integrated into clinical systems, limiting its utility for care coordination and app developers.

Additionally, challenges remain with the easy and cost-effective exchange of medical imaging.

PC-8b. What specific sources, other than claims and clinical data, would be of highest value, and why?

Patient-sourced data (such as reported symptoms, preferences, and home monitoring inputs) and data from community-based organizations (CBOs) addressing social needs would offer high value by providing critical context beyond traditional clinical and claims data. These sources can enhance personalized care, improve care planning, and support more effective interventions.

As mentioned, medical imaging is one of the frustration areas for both providers and patients that should be addressed. When we achieve a state where images are more freely exchanged and available in real-time, it is likely to have a dramatic positive impact on efforts to lower costs across our health system. Since EHRs typically do not store medical images themselves—these are usually managed by PACS or Radiology Information Systems—focusing efforts on those imaging systems is likely to yield the greatest benefit. Even when images can be exchanged, there are still limitations on the exchange due to the file sizes required to retain the necessary resolution for the recipient. This is a difficult challenge to overcome, as larger files present both technical and cost issues.

PC-8c. What specific opportunities and challenges exist to improve accessibility, interoperability and integration of clinical data from different sources to enable more meaningful clinical research and generation of actionable evidence?

A key challenge in integrating data from diverse sources is the lack of a universal standard for provider attribution. When data originates from multiple systems (e.g., EHRs, claims, apps), it becomes difficult to determine which provider should be credited for positive outcomes or held accountable for wasteful care, especially given the many non-clinical factors that influence outcomes. Accurate attribution is essential for leveraging integrated data to promote meaningful research and quality measurement—it should truly be one of the top priorities for CMS and ASTP/ONC, especially considering the priorities outlined by Administrator Oz.

PC-14a. What are the challenges today in getting patients/caregivers to sign up and use digital identity credentials?

Usability is a major barrier—the vast majority of patients and caregivers have limited health or technology literacy, making digital identity credentialing difficult to navigate. Additionally, inconsistent ID verification practices across healthcare organizations create confusion and hinder adoption.

PC-14c. What are the potential downsides?

Transitioning to a standardized digital identity model would require significant investment and changes in workflow by healthcare organizations. While it may enhance data security, the direct benefits to patient care remain unclear, which raises concerns about the return on effort for both providers and patients. This is another area where CMS, by controlling the incentives aimed at promoting desired behavior, will need to take the lead.

Providers

PR-2. What are obstacles that prevent development, deployment, or effective utilization of the most useful and innovative applications for physician workflows, such as quality measurement reporting, clinical documentation, and billing tasks? How could these obstacles be mitigated?

From a health IT perspective, frequent updates to quality measures and reporting expectations, often with limited implementation timelines, create challenges for development and lead to workflow disruptions. More predictable schedules and alignment across payment programs would help developers build sustainable, usable solutions that providers can more easily integrate into care delivery.

PR-3. How important is it for healthcare delivery and interoperability in urban and rural areas that all data in an EHR system be accessible for exchange, regardless of storage format (for example, scanned documents, faxed records, lab results, free text notes, structured data fields)? Please address all of the following:

PR-3c. Technical barriers to full data accessibility

The phrase “all data” needs clarification. If it refers to the full scope of Electronic Health Information (EHI), including externally referenced content, the technical barriers are significant. Much data in EHRs is either unstructured or lacks standard mappings, despite EHRs supporting structured and standardized formats. This limitation reduces its usefulness in exchanges beyond view-only access and may create burden for the recipient of the exchanged document or data. This is a universal challenge across care settings. Patient matching and identity resolution also remain persistent obstacles, even within standardized exchange frameworks. While AI holds the promise of utilizing unstructured data beyond view-only, it should not undermine the goal of documenting data in a standardized, structured format whenever possible and without imposing unreasonable documentation burden.

PR-3e. Priority level compared to other interoperability needs.

In rural areas, where patients often must receive care in multiple, disconnected facilities and have limited access to specialists, they may sometimes have to travel far from home for critical care. Efficient health data exchange is especially crucial in these situations. Ensuring data accessibility across systems is a higher priority in these environments to support continuity of care and reduce duplication during transitions.

PR-4. What changes or improvements to standards or policies might be needed for patients' third-party digital products to have access to administrative workflows, such as auto-populating intake forms, viewing provider information and schedules, and making and modifying an appointment?

The biggest obstacle to providing digital access to administrative workflows, such as enabling more widespread online scheduling, relates more to workflow considerations than to technical limitations. The standards are already in place to support these types of administrative processes and workflows. For example, FHIR R4 includes standard resources for Appointment, Patient, Encounter, and Coverage that are reasonably mature and widely adopted, which can be used to achieve the kinds of processes being discussed here.

Instead, the challenge in enabling patients’ selected third-party products to facilitate these processes arises more from the “trust” factor, as systems must allow independent applications, with which the provider organization has no direct business relationship, to write data into their systems. While this is not an insurmountable issue, it presents quite a different challenge from a trust perspective compared to allowing patients to retrieve their health information via read-only transactions. This is especially a concern for providers in our current environment of constant cybersecurity risks.

PR-6. Is TEFCA currently helping to advance provider access to health information?

TEFCA represents a significant step forward in enabling provider access to health information through a scalable, standardized framework. It offers the potential to unify stakeholders across healthcare, public health, research, and technology, improving consistency and reducing fragmentation.

PR-6a. Please provide specific examples

TEFCA facilitates interoperability among a wide range of participants and for various exchange purposes, including essential support for individual access services. It also promotes the adoption of HL7 FHIR-based APIs beyond the IHE Document Exchange API, allowing for a broader array of data-sharing use

cases to be addressed within a national framework. Although it is still early in implementation, it has established a foundation for cross-network data exchange that previously required complex, custom agreements between entities.

PR-6b. What changes would you suggest?

While the progress is real and measurable, it will take time before adoption reaches critical mass and we can see substantial returns more broadly. The availability from the EHR developer perspective is still evolving, with companies continuing to form partnerships with QHINs and deploy the TEFCA connectivity to their clients. Additionally, providers—particularly ambulatory practices and smaller, independent hospitals—are still evaluating their interest in adopting and utilizing the TEFCA functionality, given their concerns about resource expenditure and workflow disruption. Supporting other technologies beyond HL7 FHIR-based APIs and IHE Document Exchange APIs should be considered to facilitate migration into TEFCA without requiring immediate technology changes, while still leveraging essential TEFCA capabilities, such as the common agreement, trust framework, record locator services, and participant directory.

TEFCA's definition of eligible participants is narrower than HIPAA's treatment-based definition. This leaves out 10—15% of Carequality connections and creates unnecessary access barriers. Eligibility criteria should be broadened to better align with HIPAA and include entities beyond covered entities. Additionally, expanding use cases, such as for research, would increase TEFCA's utility. Participation incentives would also help accelerate adoption.

PR-6c. What other options are available outside of TEFCA?

Alternatives to TEFCA include state and local Health Information Exchanges (HIEs), patient portals, DirectTrust networks, consumer-facing apps, and interoperability linked to quality programs. However, these solutions are fragmented and require point-to-point arrangements, making them less scalable and comprehensive than TEFCA aims to be, as well as more costly when not collaborating under a common national exchange framework. Additionally, while Carequality does provide a national exchange framework, TEFCA is widely considered to enable the next level of nationwide interoperability and is seen as the successor to Carequality.

PR-8. What are ways CMS or partners can help with simplifying clinical quality data responsibilities of providers?

CMS can simplify clinical quality reporting by supporting consistent, standards-based approaches and reducing duplication across programs. Developing reliable, universal provider attribution models, as well as advancing patient identity resolution, is essential to ensure accurate quality measurement. Alignment across programs on specifications, measures to be reported on, data requirements, and submission methods would also significantly reduce burden and enable shared infrastructure investments. Right now, they are largely all disparate, with program requirements varying even within CMS's own programs, much less state or commercial payers.

Moreover, simplifying the programs by eliminating process measure requirements that offer minimal direct value to providers and fail to provide particularly useful information to CMS and other payers

would be greatly appreciated by the providers. A significant portion of the documentation burden that frustrates clinicians and their staff arises directly from data capture mandated solely, 100%, by requirements from CMS and other payers. This issue could be alleviated by CMS, significantly improving provider workflows.

PR-8a. What would be the benefits and downsides of using Bulk FHIR data exports from EHRs to CMS to simplify clinical quality data submissions? Can CMS reduce the burden on providers by performing quality metrics calculations leveraging Bulk FHIR data exports?

Bulk extraction of clinical quality data undoubtedly presents the industry with a real opportunity to reduce provider-side reporting burden. However, providers will only embrace payer-side calculations if it means no loss of the near real-time insight into their performance that they currently receive in many instances, which allows them to adjust their behavior, documentation patterns, and other factors during the reporting period so as to positively impact their incentives.

The EHR Association recently emphasized in our 2026 IPPS comments that data utility is minimized when there is a lack of clear attribution or timely transparency. It also requires standard measure definitions and data models to ensure that evaluations are consistent and fair.

We also note that providers will need to be 100% confident in the accuracy of the calculations before such an approach can be successfully rolled out widely, given the impact of the clinical quality measurement reporting on organizational revenue. We therefore suggest that providers should be given the choice to continue calculating their reports themselves, or grant that authority to CMS for a specified period, until the method is proven entirely accurate and trustworthy.

It is also important to recognize that while EHRs are certified to support HL7 FHIR Bulk Data Export, the lack of industry-wide usage, incentives for such usage, and operational experience means that uptake remains low. CMS should focus on creating real-world use cases and policy drivers that encourage adoption before expecting it to be a primary mechanism for quality reporting. Moreover, such adoption will likely take several years to achieve any critical mass after a robust incentive structure is put forward by CMS.

PR-8b. In what ways can the interoperability and quality reporting responsibilities of providers be consolidated so investments can be dually purposed?

CMS can reduce burden by consolidating and aligning quality measures across its programs. Emphasizing a narrower set of core measures would allow stakeholders to focus on testing and maturing the supporting standards. CMS should ensure consistency in value sets, data capture expectations, submission workflows, and calculation methodologies to reduce unnecessary variation between programs.

PR-8c. Are there requirements CMS should consider for data registries to support digital quality measurement in a more efficient manner? Are there requirements CMS should consider for data registries that would support access to real-time quality data for healthcare providers to inform clinical care in addition to simplifying reporting processes?

Registries should be required to provide near real-time performance feedback to providers, utilizing dashboards or other tools that support continuous improvement, rather than relying solely on retrospective reporting. Additionally, registries should adhere to open standards to ensure seamless data access and reduce integration complexity, rather than the proprietary approach taken by many of them currently.

PR-9b. How might CMS balance patient privacy with convenience and access to digital health products and services that may lead to significant improvements in health?

Privacy and trust are paramount. While a federal privacy law would provide important baseline protections, CMS can help balance privacy with access by promoting robust data security practices, requiring transparency in data use, and enforcing interoperability standards that empower patients without compromising their data.

PR-12. Should ASTP/ONC consider removing or revising any of the information-blocking exceptions or conditions within the exceptions (45 CFR part 171, subparts B through D) to further the access, exchange, and use of electronic health information (EHI) and promote market competition?

The EHR Association supports maintaining key information blocking exceptions, particularly the infeasibility and manner exceptions, because they balance broad access to EHI with secure implementation. These exceptions allow actors to exchange data efficiently without having to resort to expensive, proprietary methods. Keeping these flexibilities promotes standards-based exchange, fostering competition among digital health products and leveling the playing field, especially for those integrating with certified EHRs. Narrowing these exceptions could create barriers to interoperability and hinder innovation for smaller developers and providers.

Instead of revising or removing these exceptions, the EHR Association recommends continued ASTP/ONC guidance to ensure all stakeholders understand their application, advancing EHI access, and maintaining trust and feasibility across the healthcare delivery ecosystem.

We suggest two key policy actions to strengthen the information blocking framework:

1. Update the timeline condition under the Infeasibility Exception to better align with real-world clinical and technical circumstances. As noted in our HTI-2 comments, the rigid ten-business-day requirement does not always account for the complexities of healthcare workflows and the resolution of legitimate infeasibilities. A more flexible, context-aware approach would better serve the goals of the regulation without undermining access.

2. Do not finalize the proposed "requestor preference" condition to the Manner Exception. As previously stated in our HTI-2 comments, introducing this condition could unintentionally override reasonable technical and operational decisions made by actors in favor of arbitrary or unsupported preferences. This could lead to increased burden, unnecessary costs, and confusion, especially where the requested method of exchange is technically unsupported, insecure, or impractical.

We urge ONC to prioritize stability in the information blocking regulations while continuing to improve implementation through clear sub-regulatory guidance, rather than through disruptive changes to the foundational exceptions.

Payers

PA-5. What are ways payers can help with simplifying clinical quality data responsibilities of providers?

Payers can simplify clinical quality data responsibilities for providers by adopting a consistent set of standards for quality measure specifications, value sets, data requirements, and calculation logic. Today, variability across payers, including within CMS programs, adds unnecessary complexity for both providers and health IT developers.

Equally important is the adoption of standardized methods for provider/patient attribution. As noted in the EHR Association's comments on the 2026 IPPS rule, the absence of a universal attribution model undermines the reliability of performance measurement and complicates the use of shared, interoperable data. Alignment across CMS and private payers on attribution definitions would improve the accuracy of quality reporting and reduce administrative burden.

PA-5a. How interested are payers and providers in EHR technology advances that enable bulk extraction of clinical quality data from EHRs to payers to allow them to do the calculations instead of the provider-side technology?

While bulk extraction of clinical quality data holds promise for reducing provider-side reporting burden, its value depends on timely and actionable feedback. Providers are likely to support payer-side calculations only if accompanied by near real-time insight into their performance, enabling meaningful course correction during the reporting period, not just retrospective reporting.

The EHR Association has previously emphasized, including in our 2026 IPPS comments, that data utility is undermined without clear attribution and timely transparency. Payers must also align with CMS and other stakeholders on standard measure definitions and data models to ensure extracted data supports consistent and fair evaluations.

We also note that trust will be critical if any transition is to occur in which calculations are to be performed by CMS or other actors on its behalf. Given the impact of the clinical quality measurement reporting on organizational revenue, providers will need to be 100% confident in the accuracy of the calculations before such an approach can be successfully rolled out widely and should be permitted to perform the calculations themselves using a calculator of their choice. Accordingly, we suggest that at least for the foreseeable future, even as CMS potentially rolls out this approach, providers should be given the choice to continue to calculate themselves OR grant that authority to CMS.

Lastly, sharing underlying data with CMS for this purpose opens up the question of minimum necessary, given that "all" data on "all" patients are potentially contributed to the denominator in this model, meaning there would continue to be tension, even if the calculator is right.

PA-5b. In what ways can the interoperability and quality reporting responsibilities of providers to both CMS and other payers be consolidated so investments can be dually purposed? Are there technologies payers might leverage that would support access to real-time quality data for healthcare providers to inform clinical care in addition to simplifying reporting processes?

The key to reducing provider burden is consistency. Aligning quality measure specifications, implementation guides, value sets, and calculation methods across CMS and other payers would enable providers and health IT developers to leverage the same infrastructure and workflows across programs. Today's variation forces provider organizations to submit similar measures through different formats and technologies, driving inefficiency.

The EHR Association also recommends a phased, consensus-driven approach, beginning with pilots focused on a small number of core quality measures. This would allow stakeholders to test real-world workflows, evaluate interoperability standards, and iterate before scaling across the ecosystem.

Lastly, we note that the quality and integrity of the data, as captured by clinicians and their staff, are fundamental to the value of health data exchange and the information that gets reported through quality measurement. Even the best standards-based technologies, the most robust HIE infrastructure, and widespread use of AI will not yield optimal results without clear, consistent documentation. We encourage CMS to evaluate how it can influence the adoption of best practices within real-world data that will most effectively impact care and care coordination.

Technology Vendors, Data Providers, and Networks

TD-1. What short-term (in the next 2 years) and longer-term steps can CMS take to stimulate developer interest in building digital health products for Medicare beneficiaries and caregivers?

In the short term, CMS can stimulate developer interest by promoting the adoption of standardized APIs that enable seamless integration with Medicare data systems. Streamlined and transparent regulatory requirements would also lower barriers to entry for new digital health products targeting beneficiaries and caregivers.

Longer term, CMS should implement reimbursement models that incorporate digital health products into care delivery and quality programs. This includes end-to-end alignment, establishing certification requirements and exchange standards for all involved parties (not just EHR developers and providers), along with downstream incentives for consistent use by providers, payers, public health agencies, and others.

TD-2. Regarding CMS Data, to stimulate developer interest--

TD-2a. What additional data would be most valuable if made available through CMS APIs?

CMS can increase developer engagement by making provider directory data available via API and enabling real-time access to cost data. Today, cost scores tied to APMs are not visible until long after the performance periods end, which is a disincentive for some possible participants. Developers could build

more effective tools if CMS made available timely, patient-specific cost data to support both provider performance and price transparency for patients.

TD-2b. What data sources are most valuable in addition to the data available through the Blue Button 2.0 API?

Beyond Blue Button 2.0, data sources supporting provider/patient attribution are critical. Attribution inconsistencies impede accurate performance measurement and data usability. Clear, accessible attribution data would improve both developer functionality and provider trust in CMS data.

TD-2c. What obstacles prevent accessing these data sources today?

One of the primary obstacles to accessing key CMS data sources is the inefficiency and opacity of managing attribution lists with payers. The current process is burdensome for providers and does not consistently deliver value. It remains unclear whether these lists generate meaningful information for CMS or actionable insights for providers. Across both legacy formats (such as CCFL files in BCDA) and newer FHIR-based approaches, challenges with data completeness, consistency, and standardization persist, undermining trust and usability.

We encourage CMS to allow flexibility in how solutions are implemented by providers. Prescriptive requirements that mandate too explicitly how APIs need to be accessed or used can restrict the best way to present and utilize the received data. Health IT developers should have room to innovate on how they ingest the data received from the API and provide benefit to the providers in their patient-focused workflows, analytics, etc.

TD-2d. What other APIs should CMS and ASTP/ONC consider including in program policies to unleash innovation and support patients and providers?

CMS should prioritize the development of APIs that enable timely, actionable access to both cost and quality data. Providers currently lack real-time visibility into CMS-calculated cost scores, which are typically made available more than a year after the end of the performance period. This delay significantly undermines the ability of providers and developers to implement responsive, cost-effective solutions. Making cost data available through APIs would allow the industry to design tools that support proactive financial stewardship—a key objective in CMS's value-based care strategy.

In parallel, the EHR Association has long emphasized the importance of real-time quality feedback loops. Many of our member companies support dashboards within their EHRs that allow providers to monitor quality measure performance throughout the reporting period, not just retrospectively. As CMS and payers shift toward bulk FHIR and multi-source data aggregation, it is critical that providers retain the ability to track and interpret their performance in near real time.

TD-3. Regarding digital identity implementation:

The EHR Association requests clarification regarding the intended scope of digital identity implementation: specifically, who the credentials are intended for (e.g., patients, caregivers, providers, proxies) and how CMS and ASTP/ONC expect them to be used across different workflows and systems.

Greater specificity will help ensure alignment across technology developers and stakeholders and avoid misaligned implementation expectations.

TD-3a. What are the challenges and benefits?

Digital identity credentials can improve security and trust, especially by reducing the risk of unauthorized access via high-risk workflows like call center-based customer service. They also support consistent identity verification across disparate systems, potentially enhancing interoperability and patient safety.

However, implementing digital identity at scale would involve significant workflow and integration complexity, particularly for organizations that must manage identity across internal systems and external applications. A key challenge is establishing cross-organizational trust, including how third-party apps and healthcare entities verify and accept credentials without pre-existing relationships to properly match and link them to the actual records. Without transparent governance, implementation standards, and designated use cases, widespread adoption remains difficult.

TD-4. How can CMS better encourage use of open, standards-based, publicly available APIs over proprietary APIs?

CMS can better encourage the use of open, standards-based APIs by ensuring that standards are required on all sides of the exchange, including payers, public health agencies, and other stakeholders (such as labs), not just healthcare providers and EHR developers. Requiring standards only in one direction creates asymmetry, limits interoperability, and discourages investment in scalable solutions.

Additionally, the EHR Association urges CMS and ASTP/ONC to refrain from incorporating standards into program requirements until they are sufficiently mature and clearly tied to well-defined, real-world use cases. Standards must be thoroughly tested and trusted before being mandated for widespread use. Developers are willing to adopt standards the market demands, but premature adoption (as has been required in recent years) without validation and operational readiness undermines confidence and increases burden.

Finally, to support broader uptake, CMS should streamline its process for evaluating how standards are implemented and used, while avoiding imposing obligations that exceed what systems actually require for effective exchange. If the goal is meaningful adoption of common standards, the programs driving adoption must be practical, predictable, and appropriately scoped.

TD-5. How could a nationwide provider directory of FHIR endpoints improve access to health information for patients, providers, and payers? Who should publish such a directory, and should users bear the cost?

A nationwide provider directory of FHIR endpoints would significantly enhance access to health information by streamlining connectivity and reducing the friction associated with onboarding for patients, providers, and payers. To ensure accuracy and sustainability, the directory should be maintained by entities with a vested interest in its completeness and accuracy, such as those facilitating exchange (e.g., TEFCA participants) or those involved in provider credentialing and licensing (e.g., state licensing boards). Such a directory should not be limited to FHIR endpoints only, but rather any relevant endpoint or addresses used for standards-based exchange, e.g., DirectTrust addresses.

The EHR Association does not support shifting the cost burden to users. Maintaining a functional, national directory should be considered core infrastructure and supported through federal investment or aligned with existing regulatory programs.

TD-6. What unique interoperability functions does TEFCA perform?

TEFCA's unique value lies in its potential to standardize and scale nationwide health information exchange by establishing a clear governance framework, defining use cases, and maintaining consistent technical requirements across these use cases while being able to tailor these requirements to the specific needs of each use case. The EHR Association has consistently supported TEFCA's goals of reducing fragmentation in exchange models and expanding participation among a broader range of stakeholders, including providers, consumers, payers, public health agencies, researchers, and health IT developers.

One of TEFCA's distinguishing features is its encouragement of the adoption of existing standards, including IHE Document Exchange APIs and HL7 FHIR-based APIs. It employs record locator services that enable identification of all of a patient's data holders, thereby helping organizations and consumers communicate using uniform data structures and reducing the burden of managing proprietary or non-standard connections.

Importantly, TEFCA offers a centralized alternative to costly, siloed HIE participation, which may improve cost efficiency for providers, especially those currently paying to connect with multiple regional networks. If TEFCA can truly deliver on scalable, standards-based exchange, it could reduce duplicative infrastructure investments across the ecosystem.

The EHR Association views TEFCA as a promising complement, not a replacement, for other forms of interoperability, and continues to encourage CMS and ASTP/ONC to ensure its implementation is inclusive, technically feasible, and driven by mature standards.

TD-7. To what degree has USCDI improved interoperability and exchange, and what are its limitations?

The EHR Association views USCDI as a significant step forward in advancing interoperability and standardization across health IT systems. By defining broad, consensus-driven data classes and elements, USCDI establishes a common foundation for the exchange of clinically meaningful data via recognized standards such as HL7 FHIR and HL7 CDA (C-CDA). This has improved the consistency and usability of health information at the point of care and across care transitions.

However, several limitations remain:

- Limited scope: USCDI does not yet cover all EHI, though this can be addressed by incrementally expanding its content. To increase USCDI's scope efficiently and thoroughly, more modularity and stratification of certification criteria is required (see point below).

- Inconsistencies with HL7 standards: These create implementation challenges. Aligning USCDI more directly with relevant HL7 Implementation Guides would improve technical consistency.

- Update frequency: The cadence of USCDI updates can be difficult for developers and implementers to track and support, particularly when incorporated into certification and program requirements without adequate lead time. We suggest updates no more frequently than every 24 months.

- Lack of modularity: The "all-or-nothing" approach to adopting each version of USCDI presents challenges for specialty developers. For instance, data elements such as Occupation and Occupation Industry may not be applicable to pediatric-focused EHRs. This issue could be addressed by introducing stratified or modular implementation requirements within certification criteria. This would further facilitate the inclusion of health IT beyond EHRs to be certified for the data they manage, which does not necessarily encompass all of USCDI, and would reduce regulatory burden for specialized health IT.

TD-7a. Does it contain the full extent of data elements you need?

USCDI does not yet encompass the full range of data elements required to support all interoperability use cases. However, the ongoing expansion process through ONDEC offers a valuable pathway for gradually broadening its scope to provide a more complete, standardized basis for all electronic health information EHI.

The proliferation of separate USCDI+ publications has introduced confusion. The EHR Association recommends consolidating USCDI and USCDI+ into a unified standard, with use-case-specific annotations to identify critical elements for different programs. This would reduce complexity and enable health IT developers and providers to prioritize support based on their specific needs, while maintaining consistency within the broader data framework. Such consolidation must be combined with modular certification, as described in TD-7 above, ensuring that Health IT is certified only to the subset of USCDI that they manage.

TD-7b. If not, is it because of limitations in the definition of the USCDI format or the way it is utilized?

The primary limitation of USCDI is its scope, not the format itself, although we have suggested in other responses and discussions with ASTP/ONC that they should require the use of HL7 FHIR to express the data classes and data elements. This would reduce ambiguity in its translation to supporting standards, where HL7 FHIR is the standard to address first, followed by HL7 CDA C-CDA and others. Expansion through ONDEC is helpful, but the lack of clarity on USCDI's long-term purpose creates uncertainty. If the goal is to standardize all EHI under a common framework in the long term, that intent should be clearly stated and supported with a deliberate, phased strategy.

TD-7c. If so, would adding more data elements to USCDI add value or create scoping challenges? How could such challenges be addressed?

Adding new data elements inevitably introduces scoping and implementation challenges, but it is necessary for progress. Continued expansion is essential to standardize healthcare data in a way that promotes consistent exchange and meaningful use. These challenges can be mitigated by incorporating a modular approach within the Certification Program, allowing developers and providers to support only those data elements that align with their workflows or specialties.

The timing of USCDI versioning also creates challenges and could be improved upon. USCDI versions are published annually in July, but the corresponding HL7 implementation specifications (HL7 CDA C-CDA and HL7 FHIR US Core) are not finalized until May or June through the next HL7 update, ballot, and publication process to address any new data in scope. This lag complicates adoption planning. Bringing HL7 FHIR directly into the USCDI specification, as indicated above, could help align release timelines and improve predictability for implementers.

TD-8. What are the most effective certification criteria and standards under the ONC Health IT Certification Program?

Several certification criteria under the ONC Health IT Certification Program have had a meaningful impact on interoperability and trust:

- Privacy and Security (P&S) criteria have been foundational, providing a consistent baseline for safeguarding health information and serving as the closest operational proxy to HIPAA Security Rule certification.

- HL7 CDA C-CDA-based criteria and direct messaging played a critical role in advancing early interoperability. However, as the industry shifts away from document-based exchange, the program can begin focusing on modernizing these criteria toward HL7 FHIR-based API exchange.

- Public health criteria, especially for immunization and electronic laboratory reporting, have supported a scalable reporting infrastructure. However, their effectiveness is limited by the lack of reciprocal obligations on public health agencies (PHAs) to adhere to the same standards, which leads to fragmentation and unnecessary work to connect. Migration of existing, mostly HL7 v2-based, APIs to HL7 FHIR-based APIs should be considered on a use-case basis. Those in need of more comprehensive clinical data, such as Syndromic Surveillance, should migrate sooner. In contrast, those already migrating should have adequate transition time (e.g., eCR using both HL7 CDA and HL7 FHIR-based eICR reports). Others could continue to progress for the foreseeable future, including more minor updates, using HL7 v2-based APIs. New use cases, such as queries by public health to providers through TEFCA or advancing operational reporting (like bed capacity, hospital respiratory data, and operational data for emergency situations), or reciprocal data beyond the data currently shared from public health to providers and consumers, are well-suited to start with HL7 FHIR-based APIs.

- In recent years, HL7 FHIR-based API criteria, including patient and population services, have been among the most impactful, supporting ecosystem-wide data access, app development, and alignment with CMS programs.

The EHR Association encourages ONC to continue refining the certification program to support modern, scalable, and bidirectional interoperability, with increased accountability across all participants in the data exchange ecosystem.

TD-9a. What are the benefits of redefining certification to prioritize API-enabled capabilities over software functionality?

Redefining certification to emphasize API-enabled capabilities, especially HL7 FHIR-based APIs, is the right direction for modernizing interoperability. Prioritizing HL7 FHIR-based APIs over IHE Document Exchange APIs as a direction enables more flexible and efficient data sharing, allowing developers to design systems that meet their customers' specific needs, covering the spectrum of sharing individual data, datasets, and still documents. However, certain non-API-specific criteria, such as functionality-based privacy and security, remain critical and should be preserved even as certification evolves.

TD-9b. What would be the drawbacks?

A narrow focus on HL7 FHIR-based APIs may be limiting. For example, HL7 v2 APIs remain highly effective for many public health workflows, and there's little benefit in forcing migration away from proven infrastructure without a clear ROI. Moreover, certification scope must be considered holistically: replacing several low-burden attestation-only criteria with a single complex API requirement (especially tied to immature standards) can lead to a greater net burden and unnecessary expenditure. Certification reforms must consider cost, maturity, and implementation feasibility, not just the number of criteria.

TD-9c. How could ASTP/ONC revise health IT certification criteria to require APIs to consistently support exchanging data from all aspects of the patient's chart (for example, faxed records, free text, discrete data)?

HL7 FHIR-based APIs already support much of the unstructured data referenced, such as faxed or scanned documents and free-text content, through the DocumentReference resource. Expanding USCDI to more explicitly include these data types and establishing clearer expectations for supporting unstructured content would help advance interoperability.

However, it is essential to remember that for interoperability to be effective, the data must be meaningful and relevant. While unstructured data can be valuable, applying standards and structure whenever possible enhances its usability and value. We should focus not only on enabling exchange but also on improving data quality and standardization before promoting the exchange of all available information indiscriminately.

TD-9d. What policy changes could CMS make so providers are motivated to respond to API- based data requests with best possible coverage and quality of data?

The use of and responses to API-based data requests offer clear value, but given the amount of staff work and training required, provider adoption depends on meaningful incentives. While Information Blocking regulations create a compliance framework, there is little tangible reward today for providers who deliver high-quality, comprehensive API responses. CMS could strengthen this through incentive alignment—for example, rewarding automated case reporting via eCR to replace manual submission burdens. Ultimately, policy strategies should be co-developed with providers, who are best positioned to advise on how to align API engagement with clinical workflows and resource constraints.

TD-9e. How could EHRs capable of bulk data transfer be used to reduce the burden on providers for reporting quality performance data to CMS? What capabilities are needed to show benefit? What concerns are there with this approach?

EHRs with bulk data export capabilities can reduce provider burden under a digital quality measurement (dQM) model, but only if paired with bi-directional feedback mechanisms. As noted in the EHR Association's 2026 IPPS comments, providers are accustomed to reviewing performance data through EHR-based dashboards. If CMS or another entity carries out centralized measure calculation, providers must still have near real-time visibility into those results within their EHR environment. In those situations, CMS should adopt a standardized API mechanism for pushing calculated results back into EHR dashboards to ensure continuity. Additionally, we request that CMS clarify the timing, frequency, and format of such feedback to support effective care and improvement efforts.

TD-11. As of January 1, 2024, many health IT developers with products certified through the ONC Health IT Certification Program are required to include the capability to perform an electronic health information export or "EHI export" for a single patient as well as for patient populations (45 CFR 170.315(b)(10)). Such health IT developers are also required to publicly describe the format of the EHI export. Notably, how EHI export was accomplished was left entirely to the health IT developer. Now that this capability has been in production for over a year, CMS and ASTP/ONC seek input on the following:

TD-11a. Should this capability be revised to specify standardized API requirements for EHI export?

Yes, the EHI Export capability should be revised to include standardized API requirements. In its current form, codified at 45 CFR § 170.315(b)(10), EHI Export has not met its intended goals of simplifying patient access to their complete electronic health information or streamlining provider transitions between health IT systems. There are numerous variables contributing to this, but chief amongst them is the lack of defined, commonly accepted standards for both data format and delivery mechanisms. While developers are required to document their export specifications publicly, healthcare data remains highly complex and difficult to interpret consistently without a shared, well-understood standard, regardless of how detailed and thorough a developer's format specification documentation is.

This is especially true for population-level exports, where the volume and variability of data make it unrealistic to assume a seamless, low-burden export and import process. Even with thorough documentation, significant human intervention is typically required to process such exports meaningfully.

The EHR Association recommends re-scoping the EHI Export criterion to focus not on "all EHI" but instead on all USCDI data and reference guidance on how to support EHI Export using HL7 FHIR Bulk Data that includes an organized USCDI data set. We suggest that EHI Export be repurposed considering these realities and scope to a HL7 FHIR-based data set that can be shared using HL7 FHIR Bulk Data Access for a single or multiple patients that contains all USCDI scoped data as expressed in HL7 FHIR US Core/ That can grow not only based on HL7 FHIR US Core expansion (which is only a limited data set) but on an identified combination of HL7 FHIR US Core and profiles from implementation guides reflecting specialized data such as full laboratory results, care plans, and other full data sets. Any data not available in FHIR would inherently require more effort by the receiver and should not be expected to flow seamlessly under a standardized export.

Additionally, it should be clarified that EHI Export is not appropriate as a standalone solution for health IT-to-health IT migration, which often requires tailored workflows and complex mappings beyond a universal dataset. In effect, EHI Export should be repositioned as a FHIR Bulk Data use case with a

defined implementation guide, rather than an open-ended export of all possible EHI. Repurposing the EHI Export into an HL7 FHIR Bulk Data Export-based criterion will also allow for the removal of the bulk data requirements from 45 CFR § 170.315(g)(10), consolidating bulk data capabilities into a single, modular requirement that can be met by the appropriate health IT best suited to support such bulk queries. We recognize the concerns regarding EHR performance for certain bulk data volumes when implemented. A modular approach enables health IT to support the requirements, which could involve the EHR for specific bulk data use cases while not necessitating it for others, thus facilitating the optimal use of the technology necessary for these scenarios. Furthermore, it promotes the advancement of use case-specific implementation guides that address specific requirements and capabilities, including relevant Service Level Agreement (SLA) measures where necessary.

TD-11b. Are there specific workflow aspects that could be improved?

The challenges associated with EHI Export are not rooted in workflow execution. In fact, the workflow for initiating and performing an export should remain at the discretion of the developer, as it will always work most effectively when aligned with how each product is designed and used in practice. Standardizing workflow details across developers would create unnecessary rigidity and complexity without adding meaningful value. The focus of certification should remain on ensuring the capability exists and functions reliably, rather than dictating the internal steps or user interfaces used to perform the export.

TD-11c. Should CMS consider policy changes to support this capability's use?

EHI Export capabilities are triggered by specific, infrequent use cases, such as a patient requesting a computable copy of their data or a provider transitioning to a new EHR. Therefore, CMS should not mandate their use. If existing standardized exchange mechanisms (such as FHIR or C-CDA) fulfill the vast majority of real-world needs more effectively, there is no benefit in requiring the use of EHI Export for its own sake.

Instead, CMS should support the broader adoption of standards-based exchanges (e.g., HL7 FHIR and HL7 CDA), which already satisfy most interoperability needs. For exceptional cases requiring more comprehensive datasets, custom approaches remain viable until USCDI or FHIR profiles are mature enough to support “all EHI” use cases fully.

TD-13. What new opportunities and advancements could emerge with APIs providing access to the entirety of a patient's electronic health information (EHI)?

APIs that provide access to a patient’s complete EHI open the door to several key advancements. Patients and their caregivers can become more engaged partners in their care, equipped with comprehensive data to support informed decision-making. Care coordination among various providers can be significantly improved, as complete longitudinal health records become more accessible across systems. Researchers and public health agencies can also benefit, gaining access to richer, real-world datasets for analytics, population health management, and disease surveillance. These capabilities, paired with strong privacy protections and standardized data structures, represent a significant opportunity to transform health data into actionable intelligence.

In the CMS RFI listening session, however, the shortcomings of the current incentives—specifically those related to interoperability at large and bulk FHIR-based data movement more narrowly—were identified as a significant barrier to a real-time, connected healthcare system. Therefore, we also encourage CMS to provide more incentives for using this capability, which would clarify to providers why they should implement it and truly invest resources into maximizing its potential functionality. Currently, many providers claim they are unclear about why they should utilize the functionality, but this could be addressed by CMS through payment models or conditions of participation.

TD-13b. What are the primary tradeoffs between USCDI and full EHI, especially given more flexible data processing capabilities today?

The primary trade-off between USCDI and full EHI lies in striking a balance between value and complexity. USCDI defines a structured, consensus-driven subset of health data that is standardized, broadly applicable, and readily interoperable. In contrast, the full scope of EHI encompasses vast and often unstructured or proprietary data elements that vary widely across systems and use cases. While it is true that some of this data may offer unique insights about the patient or meet edge-case needs, it often lacks standardization, making it more challenging to exchange, interpret, and use effectively. Processing full EHI requires more sophisticated tools, resources on both sides of the exchange, and safeguards, which can introduce cost, workflow burdens, and risks of inaccurate interpretation of patient data without a clear value proposition for most stakeholders. Therefore, prioritizing the continued development and adoption of standards, while incrementally expanding USCDI and providing flexible pathways to access additional EHI when justified, may be the most pragmatic and effective approach.

TD-15a. How would increased use of bulk FHIR improve use cases and data flow?

The increased use of HL7 FHIR Bulk Data Export has the potential to significantly improve data flow and enable advanced use cases, such as digital quality measurement, population health analytics, and more efficient payer-provider data exchange. It provides a standardized, scalable approach for extracting large datasets, particularly useful when aggregating information across patient populations. However, realizing this promise depends on greater adoption, maturity of purpose-built implementation guides, and availability of robust testing tools. As noted in our comments on the FY2026 IPPS proposed rule, EHRs are certified to support HL7 FHIR Bulk Data Access; however, the limited deployment by providers and other stakeholders has hindered real-world experience. To bridge this gap, CMS could support targeted incentives and collaborative pilot programs that enable providers and developers to implement Bulk FHIR in specific, meaningful use cases—ultimately demonstrating its value and building industry confidence.

For example, HRSA's UDS+ initiative aimed to receive data from any FHIR-enabled system, including EHRs and specialty systems, such as dental software. While promising in theory, the reality proved more complex. HRSA's requirement for de-identified data conflicted with its expectation to receive multiple data submissions and reconcile them into a unified patient-level view. Without identifiers, accurate deduplication was impossible. The burden of aggregation was ultimately placed back on the submitting provider. This example highlights how the benefits of FHIR can be lost without aligned infrastructure, standardized deduplication strategies, and consistent support across health IT systems.

TD-15b. What are the potential disadvantages of their use?

The primary disadvantage of HL7 FHIR Bulk Data Export today is the limited real-world adoption and infrastructure maturity. Many providers and payers are unfamiliar with how to operationalize Bulk FHIR, even when they recognize the value in investing resources or effort into it, which is not always the case. The lack of implementation guides tailored to high-value use cases has also further slowed momentum. Without robust testing tools and clearly defined workflows, provider organizations may hesitate to invest in enabling Bulk FHIR, especially in the absence of regulatory or financial incentives. Furthermore, since HL7 FHIR Bulk Data Export involves high-volume data transfer, concerns arise regarding data governance, infrastructure scaling, and ensuring adequate safeguards to protect patient privacy. These challenges must be addressed before HL7 FHIR Bulk Data Export can become a reliable and widely used interoperability mechanism.

TD-17. Given operational costs, what role should CMS or ASTP/ONC or both have in ensuring viability of healthcare data sharing networks, including enough supply and demand, that results in usage and outcomes?

EHR developers can build and certify technology to support interoperability and data sharing, but that alone does not ensure adoption or meaningful use. Even when health IT developers introduce new capabilities, providers frequently do not implement them if they do not perceive sufficient value, especially when the primary beneficiaries are other stakeholders, such as public health agencies, third-party app developers, or payers. This creates a misalignment where providers bear the operational and financial burden of deploying and maintaining these capabilities, yet often receive little to no direct benefit.

To ensure the viability of healthcare data-sharing networks, CMS and ASTP/ONC must work together to create a cohesive strategy that aligns ONC's interoperability goals with CMS reimbursement models. This includes ensuring that providers are appropriately incentivized to adopt and use interoperable technologies through mechanisms such as payment reform, participation incentives, and quality program alignment.

TD-19b. Which workflows would benefit most from functional price transparency?

Functional price transparency has the potential to improve several key workflows across the healthcare continuum. One of the most impactful is medication prescribing. When providers can access real-time cost information and coverage details at the point of care, they can offer patients lower-cost alternatives, such as generic drugs or covered formulary options, based on individual insurance and pharmacy pricing. This supports more informed decision-making and greatly increases the likelihood that a patient will fill their prescription and adhere to it. In turn, this improves clinical outcomes and reduces downstream costs tied to medication non-adherence.

Without functional price transparency, providers are often unaware that a prescribed treatment may be unaffordable for the patient, which can result in abandoned prescriptions, unanticipated out-of-pocket expenses, or follow-up calls to change therapy, contributing to post-visit administrative burden. Providers are also penalized in value-based care models when patient cost barriers impede adherence,

despite having no control over drug pricing or formulary coverage. Functional transparency can directly reduce these gaps.

This potential exists in other treatment areas, of course—virtually any diagnostic process that can impose significant costs on patients but is often prescribed without clear fees beforehand.

TD-19d. What would further motivate solution development?

Solution development is often hindered not by a lack of innovation, but by the complexity and opacity of the systems involved, particularly in areas such as drug pricing, benefit design, and the variability among payer and pharmacy benefit management (PBM) systems. Developers cannot create reliable tools without stable, consistent access to up-to-date pricing and coverage data. To encourage broader solution development, CMS and ASTP/ONC can promote open, standards-based APIs for pharmacy benefit and real-time cost data, support aligned incentive models for prescribers, and continue expanding programs that demonstrate ROI for providers utilizing these tools. Reducing fragmentation and variability in this space is critical to lowering the barrier to innovation.

Value-Based Care Organizations

VB-1. What incentives could encourage APMs such as accountable care organizations (ACOs) or participants in Medicare Shared Savings Program (MSSP) to leverage digital health management and care navigation products more often and more effectively with their patients? What are the current obstacles preventing broader digital product adoption for patients in ACOs?

One significant barrier to broader adoption is the lack of interoperability and data standardization among digital health products. Without consistent and clearly defined standards for how these tools exchange information with EHRs and other core systems, providers encounter integration challenges and patients experience fragmented care. Furthermore, patients may hesitate to adopt unfamiliar tools, especially when trust is low, if they do not clearly understand how their data is protected or how use would enhance their care. Creating clearer technical and privacy standards, as well as requiring evidence of integration capabilities for digital tools supported under CMS programs, could significantly help address these issues and promote adoption across ACOs.

VB-3. What are essential health IT capabilities for value-based care arrangements?

Essential health IT capabilities for value-based care must support care coordination, performance measurement, patient engagement, and data-driven decision-making. A foundational need is standardization and consistency across CMS programs, particularly in quality measurement and reporting. Many value-based organizations operate across multiple EHR platforms and vendors, which complicates data normalization, mapping, and aggregation. Inconsistent or duplicative reporting requirements across programs create unnecessary burden and can undermine data quality. Accelerating dQM, focusing on the use of standardized data elements and formats (e.g., FHIR-based exchange of USCDI), is crucial for ensuring that health IT systems can provide accurate, near-real-time insights into quality performance.

VB-3b. What other health IT capabilities have proven valuable to succeeding in value-based care arrangements?

One often overlooked capability is bidirectional feedback between CMS and provider systems, especially for quality performance data. As EHRs shift from being the calculation engine to the data source in dQM models, providers still expect real-time visibility into how they are performing on quality measures. Establishing standardized APIs to display performance feedback from CMS or third-party aggregators within the EHR dashboard will be critical.

Furthermore, understanding and agreeing on what PHI can be used for, not only calculating measures outside the EHR but also within the provider's health IT infrastructure versus leaving the provider's infrastructure (which would include the ACO the provider may participate in) to a third-party aggregator or the payer, must be carefully considered. The PHI may exceed minimum necessary concerns where the actual measure is the minimum necessary data, not the underlying data.

Pilots like HRSA's UDS+ and CMS's Enhancing Oncology Model demonstrate the promise of FHIR-based digital quality measurement. However, they also reveal practical gaps, such as difficulty with patient attribution and deduplication across systems that do not uniformly support FHIR. These lessons reinforce the need for health IT capabilities that address real-world implementation challenges, especially where multiple systems must coordinate care for shared patients.

We also recommend CMS and ONC actively solicit provider feedback to identify which capabilities they view as most valuable and actionable. Developer capabilities are only impactful when aligned with provider priorities, workflows, and the incentives that influence their behavior, as evidenced by the mismatch in recent years between some certification criteria that developers have incorporated with minimal interest in adoption by software users.

VB-4. What are the essential data types needed for successful participation in value-based care arrangements?

Successful participation in value-based care arrangements depends on access to a comprehensive set of clinical, demographic, financial, and behavioral data, including diagnoses, medications, lab results, care plans, and claims data, which allow providers to deliver coordinated, cost-effective care. Including access to community-based care or assistance offers the most comprehensive picture. However, even the most robust datasets cannot be used effectively without accurate patient identity matching and clear attribution. These foundational elements are essential to ensuring that data is properly linked to the appropriate individuals and that providers understand which patients they are responsible for managing.

VB-5. In your experience, how do current certification criteria and standards incorporated into the ONC Health IT Certification Program support value-based care delivery?

Current certification criteria and standards under the ONC Health IT Certification Program have played a critical role in advancing interoperability and standardizing practices across certified health IT developers. This has enabled more consistent and reliable data exchange, laying a strong foundation for value-based care delivery. However, the benefits of certification are limited when other key

stakeholders, such as payers, app developers, and public health agencies, are not similarly held to the same requirements to implement a standards-based approach as EHRs. For value-based care to thrive, the entire ecosystem must be accountable to common data standards and exchange protocols, whether through certification expectations implemented by ASTP/ONC or via other regulatory levers. Without broader alignment, the burden falls disproportionately on providers and their vendors, who must navigate inconsistent data quality and innumerable, expensive proprietary interfaces across external systems.

VB-7. How can technology requirements for APMs, established through CEHRT or other pathways, reduce complexity while preserving necessary flexibility?

Technology requirements for APMs can reduce complexity while preserving necessary flexibility by aligning more closely with real-world use cases and promoting the use of implementation guides (IGs), testing tools, and reporting workflows that providers and developers find not only necessary but practical. Furthermore, encouraging the use of these resources, rather than mandating rigid implementations, can advance standardization while still promoting adoption through user-friendly approaches. Greater consistency in value sets and reporting mechanisms across programs would significantly reduce variability and burden, allowing providers to focus more on patient care and outcomes rather than navigating technical disparities. Flexibility can be maintained by supporting modular, standards-based approaches that enable providers to choose the tools that best fit their workflows, as long as they conform to a consistent underlying technical framework.

VB-8. How can other HHS policies supplement CEHRT requirements to better optimize the use of digital health products in APMs? As an example, requirements under the Conditions of Participation for hospitals (42 CFR 482.24(d)) require hospitals to transmit electronic patient event notifications to community providers. What barriers are in place preventing APM participants from receiving the same notifications?

To better optimize the use of digital health products in APMs, HHS policies should extend beyond vendor certification and address the broader ecosystem of stakeholders, especially payers, third-party application developers, and public health agencies. While CEHRT requirements have driven standardization among EHR developers, the lack of corresponding requirements for other stakeholders creates asymmetries that hinder widespread adoption and interoperability. For example, although hospitals are required under Conditions of Participation to send electronic event notifications, APM participants may not receive them if they are not connected to a network that supports those standards, or if the receiving systems lack the capability or obligation to consume the notifications.

Additionally, without reciprocal obligations and aligned incentives across all parties involved in data exchange, deployment will remain inconsistent and fragmented. Policies that create shared responsibility and accountability across the healthcare data ecosystem are essential to realizing the full value of digital tools in APMs.

VB-12. What technology standardization would preserve program-specific flexibility while promoting innovation in APM technology implementation?

To preserve program-specific flexibility while promoting innovation in APM technology implementation, HHS should invest in the development and dissemination of robust, FHIR-based testing tools. Lessons from FHIR-based pilots like UDS+ make clear that standardization alone is not enough. Robust testing infrastructure and real-world validation tools are essential. These tools would support engagement in collaborative testing environments, such as HL7 Connectathons, and provide practical mechanisms for validating data, testing real-world workflows, and applying sample data aligned with current implementation guides. Ensuring these tools are accessible and relevant not just to EHR developers but also to eligible hospitals (EHs), critical access hospitals (CAHs), physician practices, app developers, and other stakeholders would reduce implementation burden and accelerate adoption.